

MedKayfab:

Hematology Wrestling

The Bloodline Feuds



Uncle FrizzleBob's Krazy Kayfabizzaro Komik Kards
for Narrative Medical Learning Games

GW

Georg v. Westphalen

ANATOMY PUNK! ❤️ BLÖDSINN!

MedKayfab!

MedKayfab! BLÖDSINN! Call Doc Frizzle!

BLOOD FOR THE BLOOD GODS!

SOVHA! CHAOS! S!904GV!D!D



INK & GUTS!

DIY OR DIE!

FIGHT THE VIRUS!

★ SUBSCRIBE

IRON-MAN IAN (Iron Metabolism)

STAT:
Ferritin (Storage)
TIBC (Capacity)



POWER: The Hepcidin Lock. During inflammation, Ian is locked in the warehouse so Bacteria Pirates can't steal him.

LORE: Low Ferritin = Ian is broke. High Ferritin + Low TIBC = Ian is hoarding during chronic disease.

THE MACRO-TWINS (B12-Bill & Folate-Phil)

STAT:
MCV > 100
Hypersegmented



POWER: The Megaloblastic Glitch. DNA fails to replicate, so they just grow fat without dividing.

LORE: Bill needs his VIP pass to enter the Terminal Ileum. No Bill = Nerve Damage. Phil is just a leafy-green local.

SICKLE-SAM (Hbs)

STAT:
Glutamic Acid →
Valine (Pos 6)



POWER: The Vaso-Occlusive Jam. Under stress (cold, acid, low O2), Sam stiffens into a blade and clogs the highway.

LORE: Protects against Malaria (The Parasite Tax) but shreds the Spleen (Autosplenectomy) by age 10.

THAL THE GREAT (Thalassemia)

STAT:
Target Cells /
Mentzer Index < 13



POWER: The Globin Imbalance. Missing chains make the hemoglobin lopsided and highly fragile.

LORE: Crew-cut skull X-ray because his bone marrow is working overtime to fix the mess.

Stay fluffy!

NO RULES!

ANARCHY
IN THE ER

BLOOD
PUNK

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G6PD-PETE (The Oxidative Victim)

STAT:
Heinz Bodies
Bite Cells



POWER: The Fava Bean Trigger. Without his shield, Pete gets roasted by oxidative stress and sulfa-bombs.
LORE: X-linked. The Spleen-Bouncer bites out the oxidized Junk (Heinz bodies), leaving bite cells.

SPHEROCYTE SID (Spectrin/Ankyrin Defect)

STAT:
High MCHC
Perfect Circle



POWER: The Spleen Bait. Sid has no skeleton. The spleen despises perfect circles and destroys them.
LORE: High MCHC is the giveaway—he is packed too tight!

PNH-PAUL (Paroxysmal Nocturnal Hemoglobinuria)

STAT:
Missing GPI Anchor
Dark Urine



POWER: The Midnight Hit. Without his anchor, the Complement hornets assume he's an intruder and pop him at night.
LORE: Look for the morning aftermath: dark urine from nighttime hemolysis.

LEAD-PIPE LARRY (Lead Poisoning)

STAT:
Basophilic Stippling
ALAD block



POWER: The Enzyme Choke. Larry jams up ALAD and Ferrochelatase, shutting down the heme factory.
LORE: Causes sideroblastic anemia; ringed sideroblasts form as iron gets stuck in the mitochondria waiting for heme.

Stay fluffy!

NARCHY
THE ER

BLOOD
PUNK

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EPO-ERIC (Erythropoietin)

STAT:
Kidney
Messenger



POWER: The Hypoxic Scream. When O₂ is low, Eric screams at the marrow to start mass-producing red cells.
LORE: If the kidneys fail, Eric dies, and the marrow sleeps (Anemia of Chronic Kidney Disease).

HAPTOGLOBIN HARRY

(The Janitor)

STAT:
Low =
Intravascular
Hemolysis

POWER: The Cleanup Crew. Harry binds free hemoglobin to save the kidneys from toxic damage.

LORE: If Harry is entirely missing from the blood, it means there was a massive murder (hemolysis) and he used himself up cleaning the mess.



Stay fluffy!

RETICULOCYTE RICK

(The Rookie)

STAT:
>2% (Fighting)
<1% (Giving up)



POWER: The Marrow Metric. Rick is the raw recruit. If he's surging (>2%), the marrow is fighting the anesias.
LORE: If Rick is <1% during anemia, the marrow factory is broken or missing ingredients.

DIRECT COOMBS CARL

(The Detective)

STAT:
Positive =
Autoimmune
Hit



POWER: The Tag Reader. Carl detects if the immune system has placed a hit-marker on the body's own cells.
LORE: Direct Coombs looks for antibodies ALREADY attached to the RBC.



THE AGGLUTININ SISTERS

(Cold Chloe & Warm Wally)

STAT: IgM (Cold) / IgG (Warm)



POWER: The Clump. Chloe clumps in the extremities (fingers/nose). Wally attacks at core body temperature.

LORE: Chloe = Mycoplasma (Walking Pneumonia). Wally = SLE or CLL.

DIAMOND-BLACKFAN DAN

(Pure Red Cell Aplasia)

STAT: Congenital / No RBCs



POWER: The Factory Strike. A congenital defect where the red cell assembly line simply never opens.

LORE: Look for physical anomalies: triphalangeal or absent thumbs combined with pure red cell aplasia in an infant.

NEUTROPHIL NED (The First Responder)

STAT: <500 = Neutropenia



POWER: The Left Shift. When the bacterial war gets too big, Ned calls in his unbaked teenage brothers (Banda) to fight.

LORE: Ned dies fast and turns into pus. Without him, the body is a wide-open fortress.

EOSINOPHIL EDDIE (The Pink Assassin)

STAT: IgE Mediator / Parasite Killer



POWER: The Toxic Granules. Eddie unleashes major basic protein to dissolve invaders too big to eat.

LORE: The star of the NAACP (Neoplasm, Allergy, Asthma, Collagen vascular disease, Parasites).

BASOPHIL BENNY (The Rarest)

STAT: Histamine & Heparin payload



POWER: The Hidden Trigger. Benny is incredibly rare; he dumps histamine to cause extreme allergic vasodilation.

LORE: If Benny is elevated in the blood, DO NOT ignore it. Check for CML—he is often the hidden driver of the leukemia train.

LYMPHOCYTE LOU (The Special Ops)

STAT: B-Cells & T-Cells



POWER: Adaptive Memory. Lou doesn't just kill; he remembers. He holds the blueprints for lifelong immunity.

LORE: Viral infections (like EBV/Mono) cause Lou to morph into giant, atypical reactive lymphocytes.

MONOCYTE MONTY (The Long-Termer)

STAT: CD14+



POWER: The Tissue Transformation. Monty cruises the blood, but when he enters tissue, he transforms into the ultimate garbage disposal.

LORE: He eats the debris Red leaves behind and presents the evidence to the T-cells.

APLASTIC ANNIE (The Empty House)

STAT: Pancytopenia / Dry Tap



POWER: The Marrow Wipeout. The stem cells are annihilated. RBCs, WBCs, and platelets all crash to zero.

LORE: Bone marrow biopsy yields a Dry Tap (nothing but fat). Triggered by radiation, drugs, or Parvovirus B19.



MYELOFIBROSIS

MAX (The Stone Maker)

STAT:

Teardrop Cells
(Dacrocytes)



POWER: The Fibrotic Squeeze.
Max replaces the plush marrow with stiff collagen fibrosis.

LORE: Red blood cells must physically squeeze through the fibrotic stone to escape, permanently deforming them into Teardrop Cells.

POLYCYTHEMIA PAUL

(The Red Bloat)

STAT: Hct >

55% / Pruritus



POWER: The Viscous Sludge.
Too many red cells turn the blood flow into thick, dangerous mud.

LORE: Patients get intensely itchy after a hot shower (aquagenic pruritus) due to mast cell degranulation.

JAK2-JAKE

(The Constitutive Trigger)

STAT: V617F

Mutation



POWER: The Unstoppable Signal. Jake ignores the off-switch. He tells the marrow to divide forever.

LORE: The root villain behind the Myeloproliferative Wars: Polycythemia Vera, Essential Thrombocythemia, and Myelofibrosis.

ESSENTIAL TOBY

(Thrombocythemia)

STAT:

Platelets > 450k



POWER: The Platelet Paradox. Toby makes massive amounts of dysfunctional platelets, causing simultaneous clotting and bleeding.

LORE: Diagnosis of exclusion. You must rule out reactive thrombocytosis (like iron deficiency or infection) first!

BLÖDSINN!

STAT: Pseudo-Pelger-Huet / Cytopenias

A cartoon illustration of a scientist duck wearing glasses and a lab coat, sitting at a desk in a laboratory. The duck is holding a test tube and pouring its contents into a beaker. The lab is labeled 'BROKEN FACTORY!' with an arrow pointing to the background. The duck is surrounded by a mess of colorful, irregular shapes representing 'UGLY CELLS!' with an arrow pointing to them. The scene is chaotic and messy.

POWER: The Pre-Leukemic Limbo. The marrow makes cells, but they are ugly, mutated, and die before leaving the factory.
LORE: High risk of transforming directly into Acute Myeloid Leukemia (AML). BROKEN FACTORY!

STAT: Glucocerebrosidase Deficiency

POWER: The Lipid Trash Pile. Gary lacks the enzyme to break down lipids, so they accumulate inside macrophages.

LORE: Crinkled paper cytoplasm on biopsy. Severe hepatosplenomegaly and avascular necrosis of the femur.

STAT: Sky-High Ferritin / Hemophagocytosis

POWER: The Cytokine Storm. The immune system goes rogue, and macrophages begin eating the body's own healthy blood cells.

LORE: Ferritin levels explode into the tens of thousands as Hank rampages. A true hematologic emergency.

STAT: The Marrow-Booster

A colorful illustration of a witch owl with yellow eyes, wearing a purple robe and holding a wand, casting a spell over a graveyard. Three ghostly figures (two blue, one yellow) are rising from the ground. The text "MARROW MAGIC!" is written in the top left corner.

POWER: The Chemo-Recovery. Nate artificially stimulates the marrow to pump out new neutrophils.

LORE: Deployed after brutal chemotherapy regimens to pull the patient out of the neutropenic danger zone.

THE MARROW BOUNCER

(CD34+ Stem Cell)

STAT:
CD34+



POWER: The Origin Seed. Every single cell in the hematology universe starts by passing his check.

LORE: Target CD34+ cells for bone marrow transplants—they are the blank slates that rebuild the system.

BERNARD-SOULIER BEN

(The Receptor Glitch)

STAT:
Giant Platelets/
Gp1b deficiency



POWER: The Sticky Failure. Ben's platelets are huge, but they lack the receptor to grab Vinny's glue.

LOBE: Ristocetin test shows NO clumping. Unlike VWD, adding normal plasma does not fix Ben.

VON WILLEBRAND VINNY

(The Glue)

STAT:
Bleeding Time
UP / PTT UP
(sometimes)



POWER: The Platelet Bridge. Vinny connects platelets to the exposed collagen of a broken vessel wall.

LORE: Most common inherited bleeding disorder. Vinny also carries Factor VIII on his back to keep it from degrading.

GLANZMANN'S GLENN

(The Handshake Failure)

STAT:
GpIIb/IIIa
deficiency



POWER: The Aggregation Block. Glenn has no handshakes, so platelets can stick to the wall, but CANNOT stick to each other.

LORE: Bleeding time is prolonged, but platelet count is completely normal.



ITP-IZZY (Immune Thrombocytopenic Purpura)

STAT: Low Platelets / Megakaryocytes UP



POWER: The Anti-Platelet Hit. IgG antibodies mark platelets for death, and the spleen executes them.
LORE: The marrow responds by pumping out massive Megakaryocytes trying to replace the losses.

TTP-TERRY (The Terrible Five)

STAT: ADANTS13 Deficiency

FAT RN!



POWER: The Micro-Clot Storm. Huge vWF autimers don't get chepped up, acting like massive spiderwebs that shred passing red cells (Schistocytes).
LORE: Remember FAT RN: Fever, Anemia (HALLA), Thrombocytopenia, Renal, Neuro symptoms.

HUS-HUGH (Hemolytic Uremic Syndrome)

STAT: E. coli 0157:H7 / Shiga Toxin



POWER: The TesIn Shredder. Similar microthrombi to TTP, but triggered by undercooked meat toxins.
LORE: Typically hits children. Dominant symptoms are renal failure and bloody diarrhea; skips the neuro symptoms of TTP.

DIC-DAN (Disseminated Intravascular Coagulation)

STAT: Low Fibrinogen / High D-Dimer



POWER: The Paradox. The coagulation cascade fires everywhere at once, consuming all factors, resulting in massive bleeding elsewhere.
LORE: Sepsis, trauma, or obstetrical complications. PT and PTT are both elevated.

KayfaBOGGLE!
 Diognosis?!
 Diagnosis?!
 KayfaBOGGLE!

KayfaBOGGLE!

KayfaBOGGLE!
 Diognosis?!

HEMOPHILIA HARRY

(The Royal Bleeder)

STAT
Factor VIII (A)
or IX (B) /
PTT UP

POWER: The Intrinsic
Break. The intrinsic
cascade fails, preventing
a solid secondary
hemostatic plug.

LORE: X-linked recessive.
Hallmark symptom is
Hemarthrosis (deep
bleeding into the joints).
PT is normal!



FACTOR SEVEN SAM

(The Extrinsic King)

STAT
PT monitors
the Extrinsic
Pathway

POWER: The Spark.
Sam triggers the entire
clotting cascade the
moment tissue damage
exposes him to the blood.

LORE: He has the shortest
half-life of all the
factors. When Vitamin K
drops, Sam is the first to
die (PT rises first).



THE TEN-KICKER

(Common Pathway)

STAT:
Factors
10, 5, 2, 1

POWER: The Convergence.
Whether the alarm was
Intrinsic or Extrinsic,
they all meet at Factor
X to finish the job.

LORE: If the Common
Pathway is broken,
BOTH the PT and PTT
times will be elevated.



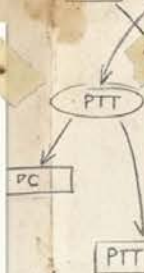
VITAMIN K-KEVIN

(The Mob Boss)

STAT:
Activates 2,
2, 7, 9, 10, C & S

POWER: The Activator.
The factors are useless,
unarmed thugs until
Kevin adds a carboxyl
group so they can bind
to calcium.

LORE: Neonates lack
gut bacteria to make
Kevin, which is why
they get a Vitamin K
shot at birth.



MEDIC-PUNK

CLOT-FIGHT!

KAYFA-BIZARRO!

BLÖDSINN!

DIAGNOSIS?!

WARRICK THE LEAKER

(Warfarin)

STAT:
Monitors with PT/INR



POWER: The Eposide Saboteur. Warrick halts the recycling of Vitamin K, slowly starving the cascade of active factors.

LORE: Because Protein C (the brake) drops first, Warrick causes a transient hypercoagulable state. Bridge with Heparin!

HEP THE HEPARIN

(The Enhancer)

STAT:
Monitors with PTT



POWER: The Immediate Halt. Hep binds to AII-III, instantly neutralizing Thrombin and Factor Xa.

LORE: Reversal agent is Protamine Sulfate. Watch out for HIT (Heparin-Induced Thrombocytopenia).

PROTEIN C & S

(The Breaks)

STAT:
Cleaves Factors V and VIII



POWER: The Regulatory Stop. They ensure a clot doesn't keep growing until it blocks the entire blood vessel.

LORE: Deficiency means the patient is chronically hypercoagulable (prone to DVTs at a young age).

FACTOR V LEIDEN

(The Unstoppable)

STAT:
Most common inherited DVT cause



POWER: The Mutant Immunity. Factor V is mutated so that Protein C physically cannot cleave it. The brakes are broken.

LORE: The clotting cascade stays OK. Suspect in young, healthy patients presenting with unprovoked DVTs/PEs.

KayfaBIZARRO!
FAKE NEWS! BUSINESS!

highly obvious plastic piastich inserted!
BLÖDSINN!

DIAGNOSIS?!

DIAGNOSIS?!

ABO-ARTHUR

(The Antigen Guard)

STAT: Types A, B, AB, O

POWER: The Universal Betrayal. O is the universal donor; AB is the universal recipient. Cross him, and you die.

LORE: Giving the wrong ABO type causes acute hemolytic transfusion reaction (preformed IgM antibodies attack instantly).



RH-RHONDA

(The Rhesus Mother)

Rh(-) Mother / Rh(+) Baby

POWER: The Second Baby Trap. Sensitisation happens during the first birth. The antibodies wait to attack the next Rh(+) pregnancy.

LORE: Give RhoGAM at 28 weeks and postpartum to hide the Rh(+) signal and prevent sensitization.



ER ADMITTING FORM

TRALI-TRICIA

(Transfusion Lung Injury)

STAT: Neutrophil lung attack

POWER: The White-Out. Donor antibodies attack the recipient's neutrophils in the pulmonary capillaries, flooding the lungs with fluid.

LORE: Non-cardiogenic pulmonary edema occurring within 6 hours of a transfusion.



TACO-TOM

(Circulatory Overload)

STAT: Volume / Diuretics

POWER: The Flood. Pushing too much blood volume too fast overwhelms a weak heart.

LORE: Unlike TRALI, TACO presents with high blood pressure, JVD, and high BNP. Treat with diuretics.



KayfaBONGO!

GW

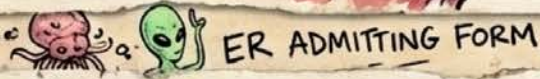
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THE SPLEEN BOUNCER (Filtration Plant)

STAT: Howell-Jolly Bodies

POWER: The Quality Control. He removes inclusions (Howell-Jolly bodies) and destroys antibody-coated cells.

LORE: If removed, patients are fatally susceptible to encapsulated Ninja bacteria (Strep pneumo, Neisseria, H. flu). Vaccinate!



PLASMINOGEN PAM (The Clot-Dissolver)

STAT: tPA target

POWER: The Demolition. When activated by tPA, Pam becomes Plasmin and digests the fibrin clot to restore blood flow.

LORE: Administered clinically (tPA) for acute ischemic strokes, but contra-indicated if the patient is bleeding!



FIBRINOGEN FRED (The Final Glue)

STAT: Factor I → Fibrin

POWER: The Hard Set. Thrombin cleaves Fred into Fibrin, turning the temporary platelet plug into a concrete wall.

LORE: Without Fred, the clot dissolves under blood pressure.



(Giving permanent marker injected retro choritis.

D-DIMER DAN (The Evidence)

STAT: High in PE and DIC

POWER: The Breadcrumb Trail. Dan only exists if a massive clot was formed AND broken down.

LORE: Highly sensitive, not specific. A negative D-Dimer rules out a Pulmonary Embolism, but a positive one doesn't confirm it.



EVGBY / PERMANENT
PAL TIME ONE MURGT!

KayfaBONGO!

GW

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What the
Kayfie FLUFF?!



PERMANENT MARKER
LETTER IN IS SHIMPUS



Permanent
note
marker
note



THE CLINICAL QUESTS: MAIN EVENTS!

THE SYMPTOM	THE SUSPECT	THE LAB CLUE	THE WINNING MOVE
The Midnight Leak: 30yo man, dark morning urine.	PNH-PAUL (The Midnight Hit) 	Negative Coombs → test. ← CD55/CD59 flow cytometry.	Identify the missing GPI anchor. Stop the Complement Hornets (Eculizumab). 
The Bridge Failure: Young girl, heavy menses, excessive mucosal bleeding.	VWF-VINNY (The Glue) 	Prolonged Bleeding Time, normal platelets? Normal PT, maybe? ? high PTT. ?	Summon Vinny with Desmopressin (DDAVP) to push glue out of vaults. 
The Joint Bleeder: Young boy, massive swollen knee (hemarthrosis) after minor fall.	HEMOPHILIA HARRY (The Royal Bleeder) 	High PTT. Normal PT, Normal Bleeding Time.	Replace the specific missing Factor (VIII or IX) to fix the Intrinsic Break. 

The Hematology Attending says: Interpret the smear, explain the PT/PTT, and find the clone. Stay fluffy!

